

CHIROPRACTIC SLEEP ACADEMY®

Foundations Workbook

An orientation to sleep & airway as structural problems.

Dr. Robert Zeravica, DC

Chiropractic Sleep Academy®

www.ChiropracticSleepAcademy.net

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HOW TO USE THIS WORKBOOK

- This workbook is an orientation, not a protocol
- It is meant to provoke structural thinking
- Answers may change as understanding deepens
- Revisit after the lecture and discussion
- Keep notes in the margins

SECTION 1

Orientation — Why This Matters

Why is sleep and airway a Chiropractic problem?

Above-Down, Inside-Out (ADIO) has been a foundational principle of Chiropractic for nearly 130 years. In everyday clinical practice, however, ADIO has most often been applied from the spine down, with pain serving as the primary point of entry. This workbook proposes an expanded framework — **ADIO from the skull down** — beginning with sleep and airway.

This perspective moves Chiropractic beyond a narrow musculoskeletal focus and toward a function-centered model that prioritizes how the body adapts, regulates, and restores itself. Using the Triad of Health concept, this approach begins with sleep and airway as expressions of **neurological and rhythmic regulation**, then considers inflammation and biochemistry, and finally addresses structural alignment.

By starting with sleep and airway, clinical evaluation shifts toward the conditions that allow the nervous system and body to recover, adapt, and coordinate function.

This workbook is designed to provide orientation to sleep and airway as structural issues, creating awareness of patterns many Doctors of Chiropractic already observe but have not yet named.

REFLECTION

What sleep-related issues do you already see in practice? Where do you feel undertrained or uncertain?

FOUNDER'S NOTE

Why CSA Exists

CSA did not begin as a business idea. It began as a pattern I could not ignore.

Early in practice, patients began reporting something unexpected — their sleep improved after their first structural adjustment. Not after weeks of care. Not after protocols. After a single structural intervention.

At first, I dismissed it as coincidence. But the feedback persisted. That feedback led me into the sleep world — conferences, conversations, and clinical environments outside traditional Chiropractic spaces. What I found was not bad medicine, but incomplete thinking. Sleep was being measured, managed, and medicated, yet the full-body biomechanics that support airway stability were largely absent from the conversation.

At the same time, I noticed something else: when Chiropractors stepped into sleep, the value of their work changed. Not because sleep was "hot," but because it was important — and under-claimed. In a field still being defined, pioneers were setting both standards and prices.

CSA exists to do two things at once

Elevate the Doctor of Chiropractic into a clinically legitimate role in sleep and airway, and address one of the most serious, under-treated problems in modern health. This Academy is not about techniques. It is about **responsibility** — and the structural thinking required to carry it.

What this workbook is designed to do

It supports three shifts that many Doctors of Chiropractic sense, but have not yet been given language for:

A mental reframe — understanding sleep not as a collection of symptoms, but as a structural and biomechanical process.

A professional reframe — recognizing that Chiropractic belongs in the sleep and airway conversation, not as an adjunct, but as a structurally competent discipline.

A career reframe — acknowledging that this work matters, remains underclaimed, and requires a higher level of clinical responsibility.

SECTION 2

The Failure of the Current Model

The issue is not bad intentions — it is an incomplete model.

Sleep is largely treated as a medical problem rather than a structural one. Airway care is fragmented across MDs, dentists, ENTs, and sleep laboratories, with each discipline addressing a portion of the problem but rarely the whole patient. As a result, no single provider consistently evaluates how structure, posture, and biomechanics contribute to sleep dysfunction.

This fragmentation leads to symptom-first thinking. Snoring is treated as noise rather than collapse. Fatigue is approached as a chemical or psychological issue instead of a mechanical inefficiency. Apnea is reduced to an event count, while the structural failure that allows the airway to collapse is often left unexplored. Symptoms are managed, but the question of *why* the airway fails is frequently overlooked.

Within this model, equipment becomes the primary solution. CPAP is commonly positioned as a default rather than a last resort. For many patients CPAP is effective, first-line medicine; the concern here is not the device but the absence of a structural evaluation around it. Oral appliances may be prescribed without a global structural assessment. While these devices can manage airflow, they do not address the underlying structure, and long-term compliance challenges are often normalized rather than examined.

A major limitation of the current approach is the lack of meaningful structural evaluation. The skull is rarely assessed as a living, moving structure. The maxillae, palatine bones, and sphenoid are often ignored. Pelvic mechanics and posture are disconnected from airway thinking. Breathing is viewed as a pulmonary process rather than a biomechanical one involving the entire axial system.

Within this environment, Chiropractic is frequently marginalized. Doctors of Chiropractic are trained to adjust, but not consistently trained to evaluate airway as a structural system. Sleep is rarely emphasized in Chiropractic education, even though Chiropractors routinely encounter patients with sleep-related complaints. Many recognize the signs but lack a framework to name and organize what they are observing, allowing referral patterns to replace responsibility.

Finally, the current model suffers from measurement blind spots. Emphasis is placed on numerical metrics such as the Apnea-Hypopnea Index (AHI), while patient experience is underweighted. Daytime fatigue, brain fog, anxiety, and overall quality of life often receive less attention. Structural change is rarely tracked, even though it may be central to long-term improvement.

REFLECTION

Where do you currently refer sleep cases — and why?

SECTION 3

Chiropractic's Missing Framework

Chiropractic has always emphasized the principle that structure governs function, yet airway has not been consistently integrated into this structural worldview. While Doctors of Chiropractic are trained to assess and influence biomechanics, airway is often approached as a localized issue rather than as a global structural system.

In many clinical settings, airway is reduced to the jaw, tongue, or soft tissues. The skull is treated as static rather than dynamic, and its relationship to cervical alignment, thoracic posture, and pelvic mechanics is rarely explored. Breathing is commonly framed in terms of respiratory chemistry, while the biomechanical coordination required to maintain airway patency is overlooked.

As a result, many Doctors of Chiropractic recognize airway-related patterns in their patients but lack a shared framework to organize those observations. Without a common language or model, clinical insights remain isolated, evaluation becomes inconsistent, and confidence in addressing airway-related issues is diminished.

What is missing is not intent or skill, but a coherent structural framework — one that connects airway, posture, and biomechanics into a unified clinical perspective.

REFLECTION

Which structures do you currently assess that may influence airway?

SECTION 4

The CSA Perspective (High Level)

The Chiropractic Sleep Academy was developed in response to the need for a coherent, structural approach to sleep and airway. Rather than offering isolated techniques or episodic training, CSA is organized as a progressive learning model that begins with foundational concepts and advances toward clinical application over time.

Within CSA, airway is approached as a biomechanical system integrated with posture, structure, and function. Emphasis is placed on understanding relationships before applying interventions, allowing clinical decisions to emerge from clarity rather than protocol.

This approach recognizes that mastery cannot be achieved in a weekend seminar. Meaningful clinical confidence develops through repetition, reflection, and refinement. CSA is intentionally designed as an ongoing educational environment, supporting Doctors of Chiropractic as their understanding deepens and their clinical reasoning matures.

REFLECTION

What would change in your practice if airway evaluation was systematic rather than incidental?

SECTION 5

Clinical Thinking Shift

When airway and sleep are viewed through a structural lens, familiar clinical findings begin to take on new meaning. Snoring is no longer understood simply as noise, but as evidence of airway instability. Fatigue is reframed from a purely chemical or psychological issue to a sign of mechanical inefficiency. Apnea is seen not merely as an event count, but as a structural collapse occurring under physiological load.

This shift in perspective alters how clinical information is interpreted. Mouth breathing may be recognized as a compensatory pattern rather than a habit. Poor sleep becomes an indicator of reduced structural adaptability. Daytime symptoms are understood as reflections of nighttime biomechanics.

As these interpretations change, so does clinical curiosity. Patterns that once appeared unrelated begin to organize around structure, posture, and function. The need for a coherent framework becomes apparent — not to replace clinical judgment, but to support it.

REFLECTION

What diagnosis or clinical assumption might you reconsider after today's lecture?

SECTION 6

Case Reflection

Consider a patient commonly seen in Chiropractic practice. They may present with chronic fatigue, unrefreshing sleep, headaches, or neck and upper thoracic tension. Snoring may be reported but is often minimized. Sleep quality is discussed briefly, if at all, and attention is typically directed toward daytime symptoms.

Despite appropriate care, progress may plateau. Symptoms improve temporarily but return. The patient continues to report low energy, difficulty concentrating, or persistent discomfort. Structural findings are present, yet their relationship to sleep and airway is not fully explored.

When viewed through a structural airway lens, this familiar clinical picture takes on new meaning. What once appeared as a collection of unrelated complaints begins to organize around nighttime biomechanics and airway stability. The clinical question shifts from *"What symptom should be addressed next?"* to *"What is failing to adapt during sleep?"*

REFLECTION

Who in your practice fits this description — including yourself?

SECTION 7

What CSA Actually Is

CSA supports a shift in clinical mindset while providing a structural framework for evaluating sleep and airway.

The Chiropractic Sleep Academy is an educational framework designed to support Doctors of Chiropractic in developing a coherent understanding of sleep and airway through a structural lens. It is not a technique, device, or protocol, and it does not replace clinical judgment or professional responsibility.

CSA emphasizes foundational concepts before clinical application. Its purpose is to organize thinking, improve evaluation, and support clinical reasoning over time. Rather than offering isolated solutions, CSA provides an environment for ongoing learning, reflection, and refinement.

The Academy is intentionally designed as a continuum. As understanding deepens, clinical confidence grows — not through the accumulation of techniques, but through clarity of perspective. CSA exists to support Doctors of Chiropractic who recognize the importance of airway and sleep and who wish to approach these issues with structural coherence and professional integrity.

CSA is intentionally quiet. Its purpose is not to compete for attention, but to support clinical responsibility.

SECTION 8

Invitation

The Chiropractic Sleep Academy provides orientation that creates awareness, a structural framework that shapes clinical mindset, and anatomical, sleep, and airway science that supports responsible application.

This workbook serves as an introduction to the concepts presented during today's lecture. It is not intended to be comprehensive, but to provide orientation and context for continued exploration.

For Doctors of Chiropractic who recognize the relevance of sleep and airway in clinical practice, and who wish to develop a structured, biomechanical approach to these issues, further education is available through the Chiropractic Sleep Academy.

Participation in CSA is optional and self-directed. It is designed for those who value foundational understanding, ongoing learning, and thoughtful clinical integration. Information regarding CSA and future educational offerings will be provided separately.

REFLECTION

What would you want to understand first if you chose to continue this learning?

Foundations Workbook (v1.1) — Contact & Further Information: Information about the Chiropractic Sleep Academy, upcoming lectures, and educational offerings will be shared separately. Professional inquiries welcome.

APPENDIX A

Foundational Diagrams

The following diagrams are visual companions to the concepts in this workbook. They are not protocols, but conceptual models intended to support structural thinking and discussion.

Figure 1 — ADIO: Skull-Down Structural Organization

The Chiropractic principle of Above-Down, Inside-Out applied from the skull down, with sleep and airway as primary expressions of neurological and biomechanical regulation.

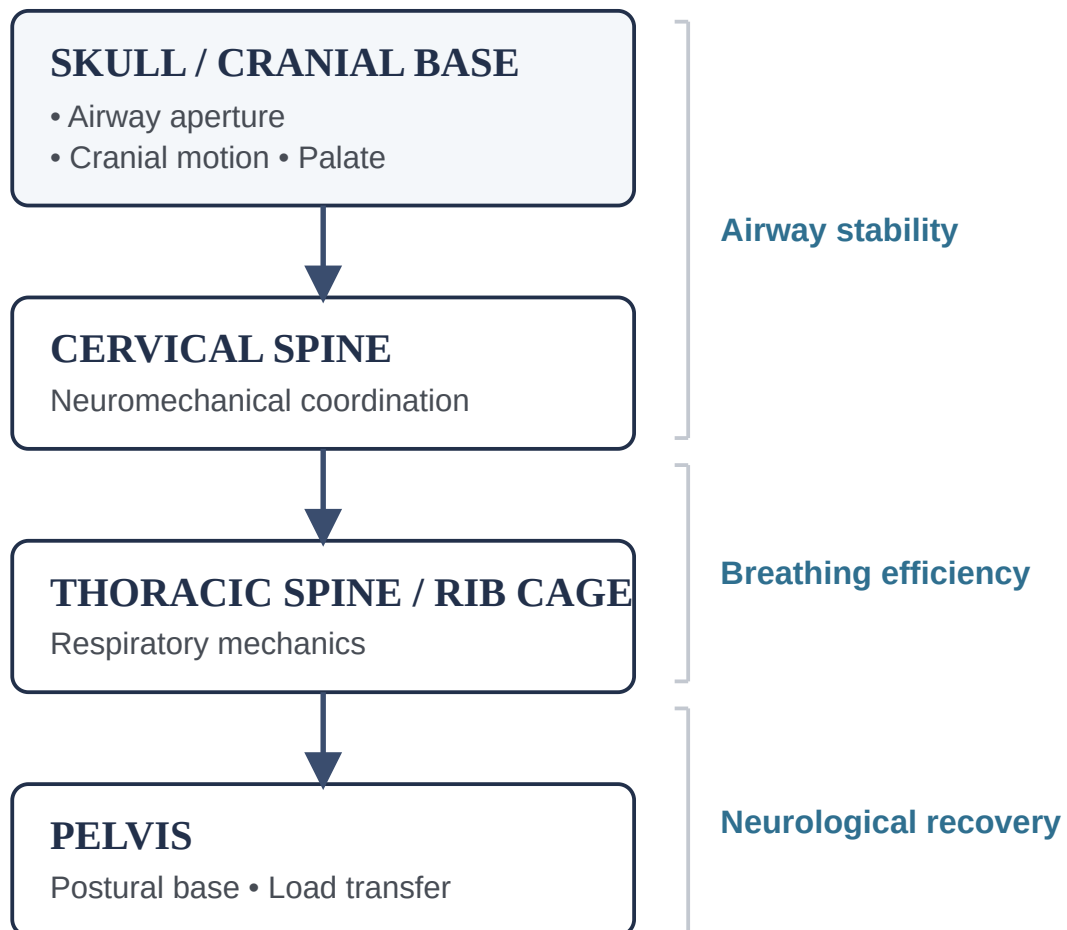


Figure 2 — The Fragmented Sleep Model

A failure of structural evaluation: current sleep treatment is fragmented and uncoupled from structural assessment.

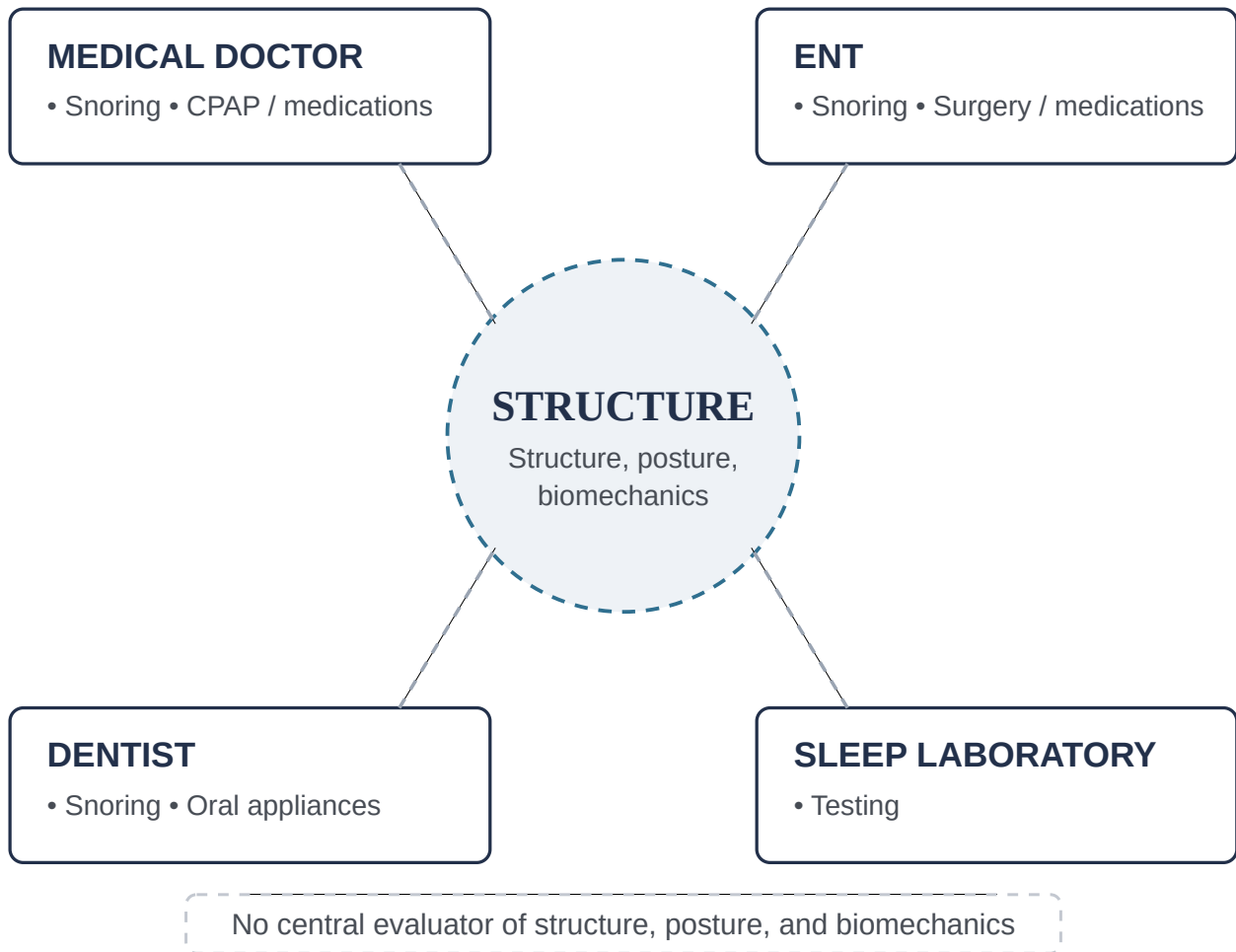
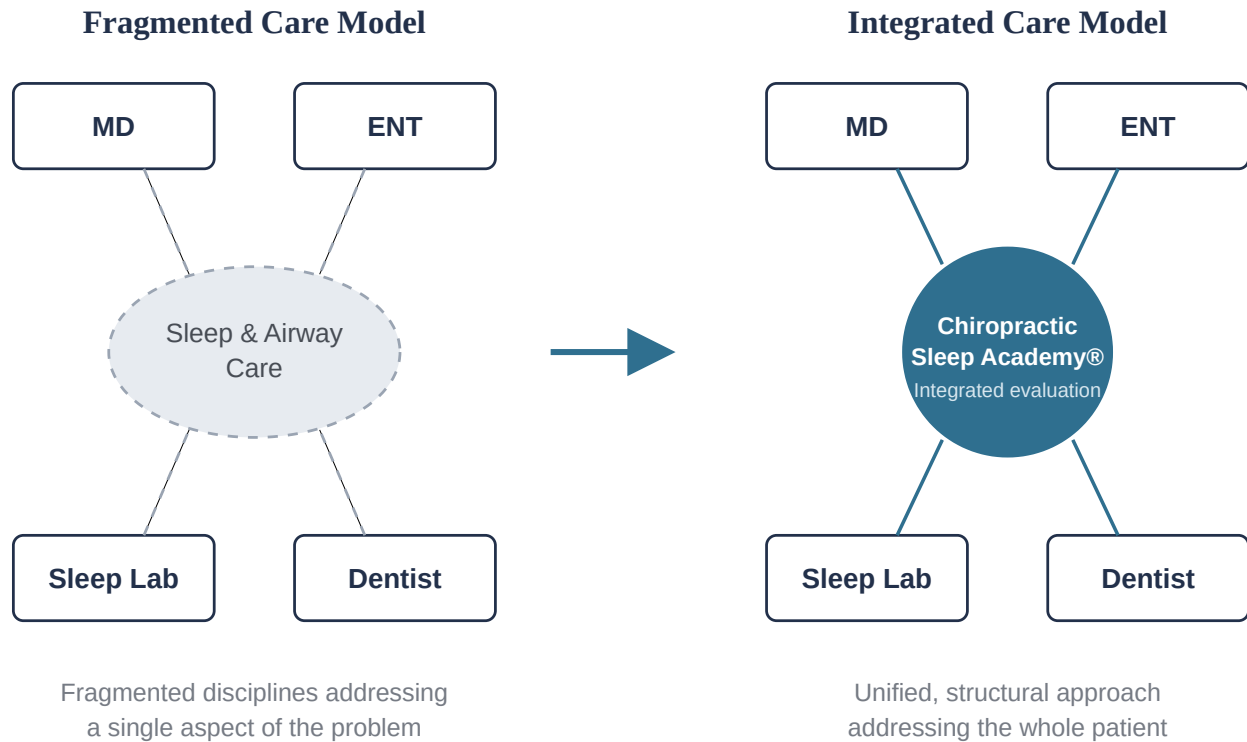


Figure 3 — Fragmented vs. Integrated Airway Care

Discipline-specific airway management contrasted with an integrated, structural approach that evaluates the patient as one biomechanical system.



CSA bridges the gaps in sleep and airway thinking.

Figure 4 — The Structural Instability Loop

How nighttime airway collapse perpetuates daytime symptoms when structural instability is left unaddressed.

